

Day 7 — Imbalance and When to See a Doctor

Module: Doṣas · **Band:** Middle · **Time:** 45 min

Learning objective

By the end of this lesson, students name three observable signs of imbalance per *doṣa*, distinguish between *lifestyle observation* (where Ayurveda’s framework can help) and *medical symptom* (where a doctor must be consulted), and articulate the safety boundary in their own words.

Materials

- The class *tridoṣa* poster
- Whiteboard, divided into TWO columns: “*Lifestyle range — observe + adjust*” and “*Medical territory — see a doctor*”
- Scenario cards (in `activities/activity-03-scope-debate.md`)

Vocabulary refreshed

- *vikṛti* (imbalance) — reused from Day 3.

Lesson flow

Warm-up (5 min)

- Recall: “*What is the difference between prakṛti and vikṛti?*”
- “*Today: when is a vikṛti just a temporary tilt — and when is it something that needs a doctor?*”

Core (20 min)

1. **The signs of imbalance per doṣa** — build the table:

Doṣa	When excess (vitiating)	Common lifestyle observations
<i>Vāta</i>	dry, cold, wind	dry skin, constipation, insomnia, anxiety, joint cracking, gas, light-headedness
<i>Pitta</i>	hot, sharp, oily	heartburn, acne, irritability, sharp anger, skin rashes, excess sweat, premature greying
<i>Kapha</i>	heavy, cold, oily	congestion, mucus, lethargy, weight gain, swelling, slow digestion, oversleeping

2. **The crucial framing** — write on the board, get students to copy:

**Some of these belong in lifestyle awareness. Some belong in a doctor’s office.
The skill is knowing which is which.**

3. **Build the two-column table together:**

Lifestyle range — observe + adjust	Medical territory — see a doctor
Mildly dry skin in winter	Skin lesion that doesn't heal in 2 weeks
Occasional acidity after spicy food	Heartburn that wakes you at night, persistent
Feeling sluggish after a heavy meal	Persistent fatigue lasting weeks
Mild congestion in monsoon	Cough lasting > 2 weeks, blood in mucus
Restless sleep one night	Insomnia > 2 weeks
Anxious before a test	Anxiety that interferes with daily life
Bowel a bit off for a day	Diarrhoea / constipation > 1 week, blood in stool
Mild headache after late night	Severe headache, recurrent, with nausea or vision change

“The left column — the framework can help. Notice patterns, adjust routine, try the opposite quality. The right column — go to a doctor. Doṣa-language is not enough.”

4. Source paraphrase:

“Knowing the doṣas in both their natural and disturbed states is the foundation of medical practice.” — paraphrased from Aṣṭāṅga Hṛdaya, *Sūtrasthāna* 1.8

Note even Vāgbhaṭa's classical statement situates *doṣa* knowledge as a *foundation* — not the whole of medicine. Diagnosis is more than naming the *doṣa*.

5. The misuse pattern to watch for:

- “It's just my Vāta acting up” — used to dismiss anxiety that needs help.
- “I'm naturally Kapha so I can't lose weight” — used as a fixed-identity excuse.
- “Eat this herb to balance your Pitta” — non-doctor giving medical advice. Refuse.
- “Don't take that medicine; it'll make your Vāta worse” — dangerous. Never substitute *doṣa* advice for prescribed treatment.

Activity (15 min) — Scope debate

Detailed in [activities/activity-03-scope-debate.md](#). Summary:

- Pairs receive a scenario card describing a person and symptom.
- Pair discusses: lifestyle range or medical territory? Why?
- 5 scenario rounds. After each, brief class share.
- Some cards are deliberately ambiguous — the discussion is the lesson.

Wrap-up (5 min)

- **Exit question** (whole class, no slip needed): *“In your own words — when do you stop using doṣa language and call a doctor?”*
- Take 3 student answers. The frame to aim for: *“When the observation lasts longer than a week or is severe, or when someone is suffering. Doṣa language is for noticing patterns over time — not for treating something that's actually wrong.”*

- Preview Day 8: “Tomorrow — you start designing your own 5-day doṣa-aware week.”

Student-facing content

The safety rule, in one sentence:

Doṣa language is for *noticing patterns* — not for *treating illness*.

When to use the framework: - Mild, brief, lifestyle-level: bit of dry skin, slow morning, late-night restlessness once. - You want to notice your patterns over weeks and months.

When to STOP and see a doctor: - Severe, persistent, or worsening symptoms. - Any symptom lasting > 1–2 weeks. - Anything that interferes with school, sleep, eating, or basic mood. - Blood, fever, sustained pain, fainting — immediately.

Homework

- Look back at your week. Was there ONE thing where you said “my X is acting up” or “I’m too tired” — that you might have ignored too easily? Write one line.

Differentiation

- **One tier up:** Imagine you are an Ayurvedic clinician AND a modern doctor in conversation. Write a 3-line dialogue about a teenage patient with persistent dry cough. Who says what?
- **One tier down:** Memorise the two-column rule of thumb: “*persistent or severe* → *doctor*; *brief and mild* → *notice + adjust*.”

Teacher notes

- This is the safety lesson. **Do not skip even if compressing the module.**
- Some students may have actual unmanaged symptoms. Be alert. Refer to school counsellor or nurse where needed.
- Don’t list specific herbs or remedies even casually. Module 4 is observation-only.
- Some Indian families do consult Ayurvedic doctors. Honour that. The point is: *a qualified clinician* — Ayurvedic or modern — is who diagnoses. *A student with a wall poster* is not.

Citation(s) used in this lesson

- Aṣṭāṅga Hṛdaya, *Sūtrasthāna* 1.8 (paraphrase). See `sources.md`.